REGULATORY DESIGNATIONS

US FDA & EMA

Insights Corner

Month: June 2026

Presenter: Team Octavus

Content

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PRIOR ISSUES

- Highlights from AACR-25 (May, 25)
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- Antibody Drug Conjugate: The next big investment in Oncology (Aug ,25)
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- Cell Therapy Within CGT (Apr, 26)
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Additional: 1 Beautiful Bill

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Executive Summary

OVERVIEW

Standard timelines are too slow for today's pipelines

For targeted therapies serving small patient subsets, the traditional development path is simply unfeasible; designations exist to provide regulatory flexibility through adaptive trial designs, faster reviews and commercial incentives

PATHWAYS

US FDA vs EMA at a Glance

US FDA is more flexible, accepting early-stage and real-world data; EMA requires larger, more rigorous studies. This divergence makes early designation planning critical for global drug developers

TRENDS

Designation Trends

ODD are currently the safest, most rewarding pathways; ODD offers near-certain retention with 7-year exclusivity while RMAT is growing fast with ~354% grant growth, reflecting rapid growth of innovative therapies like CGT

BUSINESS IMPACT

Strategic Value of Stacking

Combining ODD + BTD + Priority Review can compress time-to-market by 2-4 years, add $100M+ in lifecycle value, and boost M&A premiums by 20-40%, making designations a core business strategy, not just a regulatory step

REFORMS

2026 Reforms Reshaping the Field

The Catalyst orphan exclusivity reversal opens previously blocked rare disease markets to competition; PRV program is extended to 2029; EMA PRIME now has mandatory pre-submission checkpoints and psychedelic therapies are fast-tracked via executive order with $50M in federal funding

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Regulatory Landscape & Pathway Architecture

(US FDA & EMA Designation Framework)

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The Strategic Importance of US FDA and EMA Designations in 2026

Overview

- Designations are special statuses granted to drug candidates by regulatory authorities, such as the US FDA in the United States and the CHMP/EMA in Europe, based on their intended indications and development needs
- These designations are typically awarded to therapies targeting serious or life-threatening conditions, addressing unmet medical needs, and are intended to facilitate and expedite the drug development, review and approval processes

Navigating Regulatory Divergence

The US FDA is increasingly flexible, often accepting real-world evidence and single-arm trials for accelerated pathways. The EMA, however, generally demands larger, more comprehensive datasets with active comparators and longer follow-up periods. Designations force early alignment with these differing expectations

Complex Targeted Therapies

For highly targeted oncology pipelines such as next-generation inhibitors for EGFR-mutant advanced Lung cancer the standard 10 to 15-year development lifecycle is too slow. These therapies target specific patient subsets, meaning traditional large-scale trials are often unfeasible. Designations provide the regulatory flexibility needed to utilize adaptive trial designs and novel biomarkers

Harmonizing Cross-Border Strategy

Because the EMA requires consensus across member states, early engagement through PRIME helps sponsors structure their clinical plans. This ensures that data collected from investigators and KOLs across key European markets, such as Italy or Croatia, will hold up under centralized EMA scrutiny

US FDA Designations: An Overview

	Fast Track Designation	Breakthrough Therapy Designation	Priority Review Designation	Orphan Drug Designation	Accelerated Approval
Review Program	Designation	Designation	Designation	Designation	Approval pathway
Overview	It is a process intended to facilitate the development & expedites regulatory review of drugs, to treat serious conditions that address unmet medical need	It is a process designed to accelerate the development & regulatory review of drugs based on clinically significant endpoint	An original application or efficacy supplement for a drug treating serious condition. If approved, would provide significant improvement in safety or effectiveness	It is granted to drugs, (including biologics) intended for the prevention, diagnosis, or treatment of diseases affecting fewer than 200,000 individuals (rare disease) in US	It is a pathway allowed faster approval of drugs to treat serious conditions that fill an unmet medical need
When to Apply	With IND, or after; no later than pre-BLA or pre-NDA meeting	With IND or after; no later than the end of P2 meeting	With original BLA, NDA, or efficacy supplement	At any time in its drug development process, prior to submission of marketing application	At an early stage of clinical development
US FDA Response Timeline	Within 60 calendar days of FTD request receipt	Within 60 calendar days of BTD request receipt	Within 60 calendar days of receipt of original BLA, NDA, or efficacy supplement	Within 90 days of ODD request receipt	-
Features	Frequent meeting with US FDA, eligibility for accelerated approval & priority review, rolling review	All FTD features, intensive guidance on an efficient drug development program, organizational commitment; rolling review	Shortens US FDA application review timeline to 6 months compared to 10 months for a standard review	25% tax credits for qualified clinical trials, waiver of NDA/BLA user fees, eligibility for 7-year marketing exclusivity	Include drugs has an effect on a surrogate or an intermediate clinical endpoint that is reasonably likely to predict a drug's clinical benefit
Additional Considerations	Designation may be withdrawn if product no longer satisfies fast track qualifying criteria	Designation may be removed if product no longer satisfies BTD qualifying criteria	Designation will be granted upon filing of original BLA, NDA, or efficacy supplement	US FDA perform a distinct disease assessment based on MOA, pathophysiology, etiology, treatment options & prognosis	Requires submission of promotional materials, post-marketing confirmatory trials, expedited withdrawal if eligibility criteria not met

Abbreviations: BTD: Breakthrough Therapy Designation; FTD: Fast Track Designation; ODD: Orphan Drug Designation

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FDA Special Designations & Programs

	Regenerative Medicine Advanced Therapy Designation	Rare Pediatric Disease Designation	Qualified Infectious Disease Product Designation	Priority Review Vouchers	Real-Time Oncology Review	Project Orbis
01 Review Program	Designation	Designation	Designation	Voucher Program	Review Program	Program/Initiative
02 Overview	It provides an expedited development & regulatory review pathway for regenerative medicine therapies (cell therapy, tissue product, etc.) intended to treat serious diseases	It is a program designed to encourage the drug development to treat or prevent rare pediatric diseases	A QIDP involves the development of antibacterial & antifungal drugs for human use that treat serious or life-threatening infections	PRVs was designed to encourage development of new drug & biological products for prevention and treatment of certain tropical diseases (Malaria, Tuberculosis, Dengue, etc.	It is a program designed to streamline regulatory review process, with top-line efficacy and safety results from the pivotal clinical trial	A collaborative framework enabling simultaneous submission and review of oncology products among international regulatory partners
03 When to Apply	With IND, or as an amendment to an existing IND	Before NDA/BLA filing	With IND submission or through pre-IND correspondence	With original BLA, NDA, or efficacy supplement	With request submitted to during IND phase	With submissions of NDAs & BLAs, supplemental NDAs & BLAs with oncology indications
04 US FDA Response Timeline	Within 60 calendar days of RMAT request receipt	Within 60 days of RPD request receipt	Within 60 calendar days of RMAT request receipt	Within 60 calendar days of receipt of original BLA, NDA, or efficacy supplement	Within 20 business days of RTOR request receipt	US FDA adheres to its standard review timelines
05 Features	Include all benefits associated with fast track & breakthrough therapy designation	Sponsor may receive a PRV, that can be used to obtain Priority Review for another product or sell to another sponsor	5-year market exclusivity extension, priority review to first application submitted for QIDP approval, QIDP drug also received FTD	Voucher granted by FDA upon approval of tropical disease product, thus, allowing sponsor to receive priority review for another human drug application	To support early US FDA evaluation, improve review quality, early, iterative US FDA-sponsor communication	Earlier patient access to product in international markets to cancer treatment, establish more uniform global standards of treatment
06 Additional Considerations	US FDA will not grant RMAT if the IND is inactive or on hold	As of Feb 3, 2026, US FDA will no longer award PRVs under rare pediatric disease program after Sep 30, 2029	Applicable only to human drugs, not available for biologics or medical devices	To redeem a PRV, sponsor must inform US FDA at least 90 days prior to submission of human drug application	Voluntary program, no guaranteed early approval	Participation in Project Orbis does not affect RTOR timelines; Each regulatory agency independently makes its regulatory decisions

Abbreviations: BTD: Breakthrough Therapy Designation; FTD: Fast Track Designation; ODD: Orphan Drug Designation; PRVs: Priority Review Vouchers; QIDP: Qualified Infectious Drug Product; RPD: Rare Pediatric Disease; RMAT: Regenerative Advanced Therapy; RTOR: Real Time Oncology Review

EMA Designations: An Overview

	Orphan Drug Designation	Accelerated Assessment	Conditional Marketing Authorisation	PRIME (priority medicines)	Exceptional Circumstances
Nature of Program	Designation	Review Program	Approval pathway	Designation	Approval pathway
Overview	Orphan drug designation granted to substances intended for the treatment, prevention, or diagnosis of rare and serious conditions	Accelerated assessment reduces the review time for marketing-authorization application required by EMA CHMP	EMA grants conditional marketing authorization to medicine that addresses an unmet medical need of patients based on less comprehensive data than normally required	PRIME is an EMA initiative, facilitates the development of medicines targeting unmet medical needs by demonstrating meaningful improvement in clinical outcomes	A type of marketing authorization granted to medicines when full efficacy and safety data cannot be generated under normal conditions of use
Timeline/ Review Impact	US FDA responds within 30 days of request receipt	CHMP can reduce marketing authorization timeframe from 210 days to 150 days	Authorization valid for one year and can be renewed annually	CHMP responds within 40 days from start of the procedure	Authorization valid for five year and can be reassessed annually
Eligibility Criteria	If a drug treat, prevent, or diagnose a life-threatening disease; disease must not affect >5 in 10,000 people in EU; no satisfactory method of diagnosis, prevention or treatment exists, if method exists drug have significant benefit over existing therapies	If product is of major interest for public health and therapeutic innovation	Positive benefit-risk balance, applicant will be able to provide comprehensive data post-authorization, fulfils an unmet medical need	If drug not yet authorized in EU; no treatment option exists, or if a drug can offer major therapeutic advantage over existing treatments with request submitted to EMA during exploratory phase of clinical trial	For extremely rare indications, if comprehensive data cannot provide, data collection not possible or unethical
Additional Considerations	Scientific advice (eg: research protocol, technical assistance), market exclusivity for 10-years, fee reductions; Eligible for standard marketing authorization after submission of complete clinical data	-	Required submission of further clinical dataset in future	Potential for accelerated assessment, early appointment of rapporteur, meetings with rapporteur & experts to discuss development plan	Typically, not lead to full dossier completion & therefore not become standard marketing authorization

US FDA vs EMA: Designations & Expedited Review Programs(1/2)

	U.S. FOOD & DRUG ADMINISTRATION	EUROPEAN MEDICINES AGENCY
Rare Disease Policy		
Law	**Orphan Drug Act (1983)**	**Regulation on Orphan Medicinal Products (1999)**
Eligibility Criteria	Prevalence under 200,000 US inhabitants, low commercial potential	Prevalence under 5:10,000; expected sales of the medicine are insufficient to justify the investment required for its development
Merits	Tax credit (25%), user fee waiver, 7-year marketing exclusivity	Fee reductions, scientific advice (eg: research protocol, technical assistance); 10-years marketing exclusivity
Accelerated Regulatory Review		
Program	**Priority Review (1992)**	**Accelerated Assessment (2004)**
Eligibility Criteria	Drugs that shows significant improvement in safety or efficacy for a serious condition	Drugs that are of major public health interest and therapeutic innovation
Merits	Shortened US FDA review goal (6 months vs 10 months under standard review)	Reduced assessment timeline (150 days vs 210 days under standard assessment)
Preliminary Approval		
Program	**Accelerated Approval (1992)**	**Conditional Marketing Authorisation (2004)**
Eligibility Criteria	Treatment of serious condition, shows improvement in surrogate endpoint that is likely predict clinical benefit	Positive benefit/risk balance, comprehensive data post-authorization, satisfies unmet medical need
Merits	Approval based on surrogate endpoint or intermediate clinical endpoint with confirmatory trials conducted post-approval	Approval based on limited clinical data with additional post-authorization commitments required, valid for one year and can be renewed annually

US FDA vs EMA: Designations & Expedited Review Programs(2/2)

	U.S. FOOD & DRUG ADMINISTRATION	EUROPEAN MEDICINES AGENCY
Expedited Drug Development		
Program	**Breakthrough Therapy Designation (2012)**	**PRIME (2016)**
Eligibility Criteria	Drugs that treat serious conditions and show preliminary evidence of substantial improvement over existing therapies	Drugs that address unmet medical needs by demonstrating meaningful improvements in clinical outcomes
Merits	All fast tract benefits, meetings with senior US FDA staff, adaptive & efficient trial design	Eligibility for accelerated assessment, early rapporteur appointment, meetings with regulatory experts to discuss clinical development plan
Approval Based on Limited Data		
Program	-	**Exceptional Circumstances (2004)**
Eligibility Criteria	-	Comprehensive data cannot be obtained because of the rarity of the disease or ethical considerations
Merits		Approval for treatments that normally would not qualify for regulatory approval under standard requirements

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Impact of Designations on Drug Development and Regulatory Success

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Drug Development Timeline: Standard vs Designated Pathways

Standard	Discovery & Pre-clinical	P1	P2	P3	FDA review	Approval
Orphan Drug Designation	Discovery & Pre-clinical	P1	P2	P3	FDA review	Approval
Fast Track Designation	Discovery & Pre-clinical	P1	P2	P3	FDA review (rolling)	Approval
Accelerated Approval	Discovery & Pre-clinical	P1	P2	FDA review	P3	Approval
Priority Review	Discovery & Pre-clinical	P1	P2	P3	FDA review (accelerated)	Approval
Breakthrough Therapy Designation	Discovery & Pre-clinical	P1	P2	P3	FDA review (rolling)	Approval

Standard: Full clinical endpoints required, baseline pathway, US FDA review time: 10 months, approximate time from IND to drug approval: ~8-9 years

Orphan Drug Designation: Tax credits (25%) & grants from P1, Fee waiver, 7-year market exclusivity, US FDA review time: 10 months or 6 months if Priority Review, approximate time from IND to drug approval: ~ 7-8 years

Fast Track Designation: US FDA reviews based on P2 data while P3 runs in parallel, US FDA review time: 10 months or 6 months if Priority Review, approximate time from IND to drug approval: ~ 7-8 years

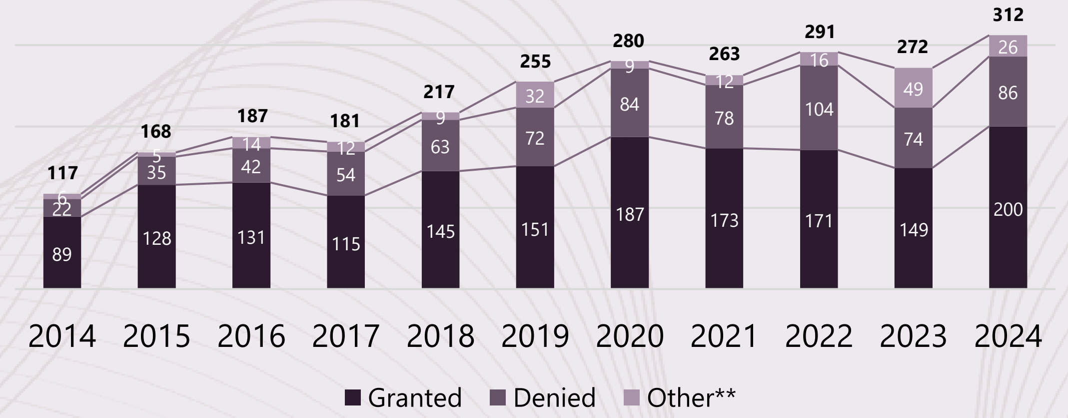
Accelerated Approval: Approval is granted on conditional basis based on surrogate/clinical endpoint of P2/ early P3, Requires confirmatory trials post approval to confirm benefits, US FDA review time: 10 months or 6 months if Priority Review, approximate time from IND to drug approval: ~ 5-6 years

Priority Review: US FDA review time: 6 months, accelerated review window to earlier US FDA action, approximate time from IND to drug approval: ~ 6.5 – 7.5 years

Breakthrough Therapy Designation: P3 adaptive/surrogate, Includes all FTD features, Intensive US FDA guidance throughout development process involving senior US FDA official, approximate time from IND to drug approval: ~ 7-8 years

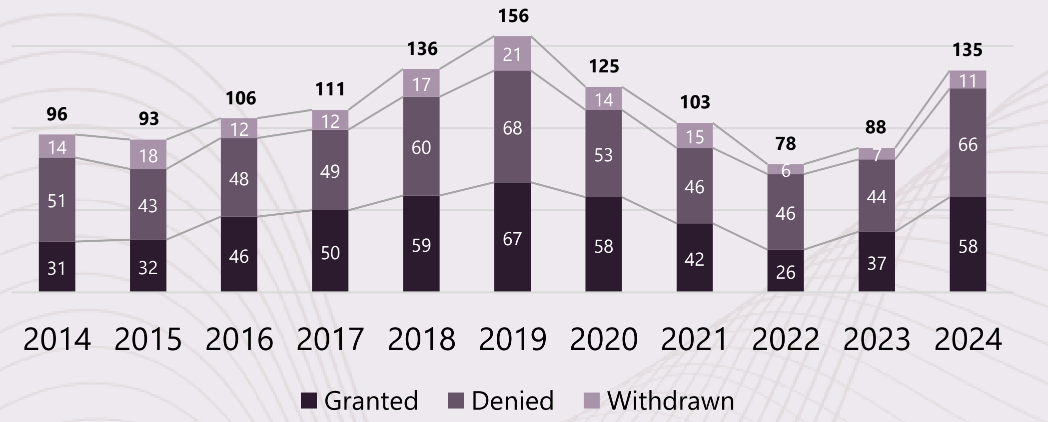
Success Rate across Regulatory Pathways(1/3)

Fast Track Designation (FTD)



Note: CDER data are reported as of 12/31 of the stated fiscal year & taken from US FDA website
** Other accounts for scenarios where the Fast Track Designation Request could not be granted or denied (Withdrawn Fast Track Designation Request, Withdrawn Application, Inactivated IND)

Breakthrough Therapy Designation (BTD)



Note: CDER data taken from US FDA website

KEY HIGHLIGHTS

- ❖ FTD demand has surged ~167% from 117 (CY2014) to 312 (CY2024) requests, while approval rates declined from 76% (CY2014) to 64% (CY2024); shows that sponsors are required to demonstrate stronger preliminary efficacy data, unmet need rationale and careful pre-submission assessment to avoid the higher denial/withdrawal rate (>30%) that is seen in recent years
- ❖ BTD remains harder to secure as across CY2014–2024 only ~41% of requests were granted (506 vs 574 denials), requests dropped ~44% from the 2019 peak before partially rebounding, and current ~47% denial rates mean it should be pursued only for assets with clearly transformative clinical data

KEY TAKEAWAY

- ❖ FTD and BTD are now in high-demand but rising request volumes alongside falling approval rates mean sponsors should only pursue these designations when they have genuinely differentiated, early clinical data and should invest heavily in pre-submission strategy to avoid costly denials

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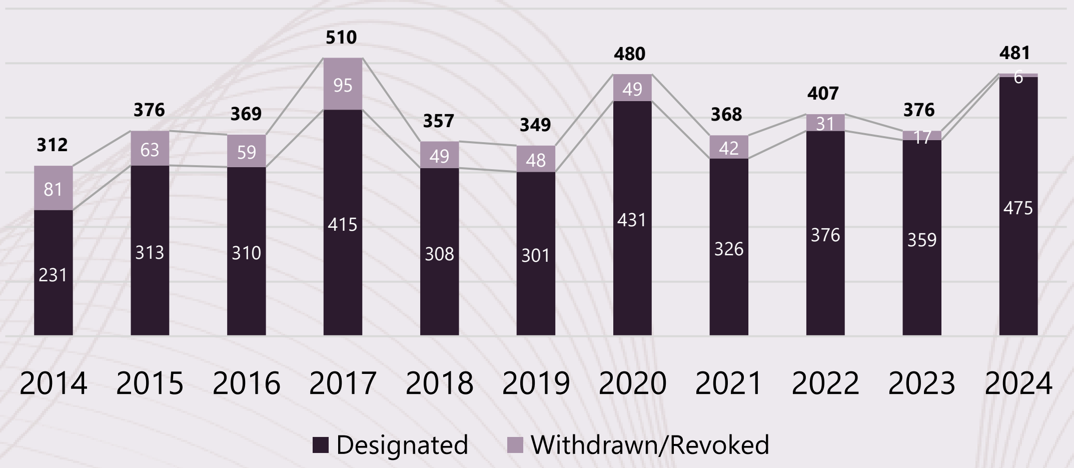
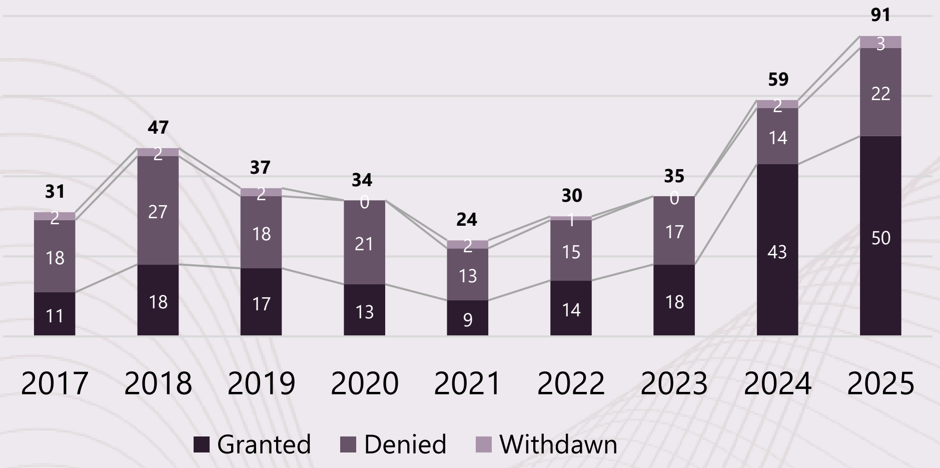
Success Rate across Regulatory Pathways(2/3)

Orphan Drug Designation (ODD)



Note: Designated also include designated/approved molecule that got designated in 2025
Data taken from US FDA ODD website

Regenerative Medicine Advanced Therapy (RMAT) Designation



Note: Cohort: December 13, 2016 - September 30, 2025; For 2025 16 requests are still pending as of Sep 2025
CBER data as of September 30, 2025; Data taken from US FDA website

KEY HIGHLIGHTS

- ODD is now a very high-yield, low-risk path as grants surged ~106% from 231 (CY2014) to 475 (CY2024) while withdrawals collapsed 93% from 81 (CY2014) to just 6 (CY2024), having ~97% program retention in recent years (2023-2024) demonstrates rare disease development as the dominant, lowest-risk pharmaceutical strategy with 7-year market exclusivity benefit
- RMAT designation achieved ~354% growth from 11 (CY2017) to 50 (CY2024) grants, with approval rates improving from ~35% (CY2017) to ~55% (CY2025) and <4% withdrawals, creating a window to secure multiple RMAT designations for regenerative products before the space becomes crowded

KEY TAKEAWAY

- Rare disease and regenerative medicine pathways (ODD and RMAT) have become the most attractive regulatory bets for innovative therapies (like CGT) as ODD now offers a high-yield, low-risk route with ~97% program retention and 7-year exclusivity, while RMAT's 354% grant growth, rising ~55% approval rate, and <4% withdrawals create a time-limited opportunity to stack multiple RMAT designations before the field saturates

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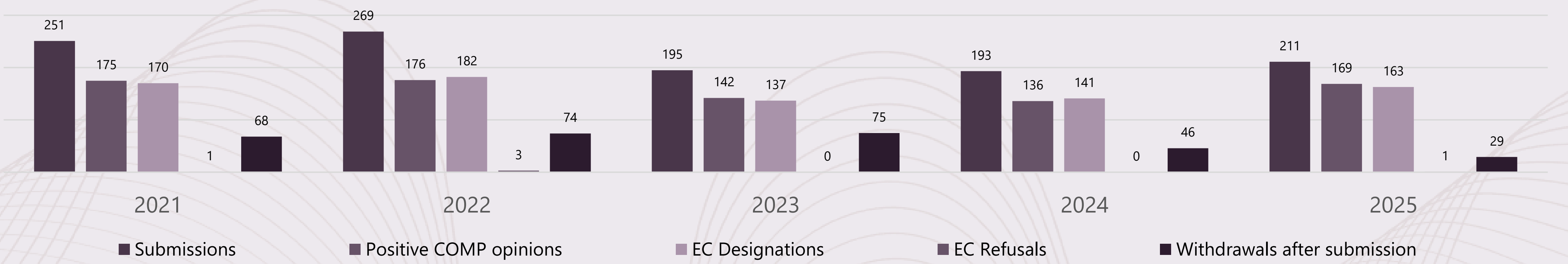
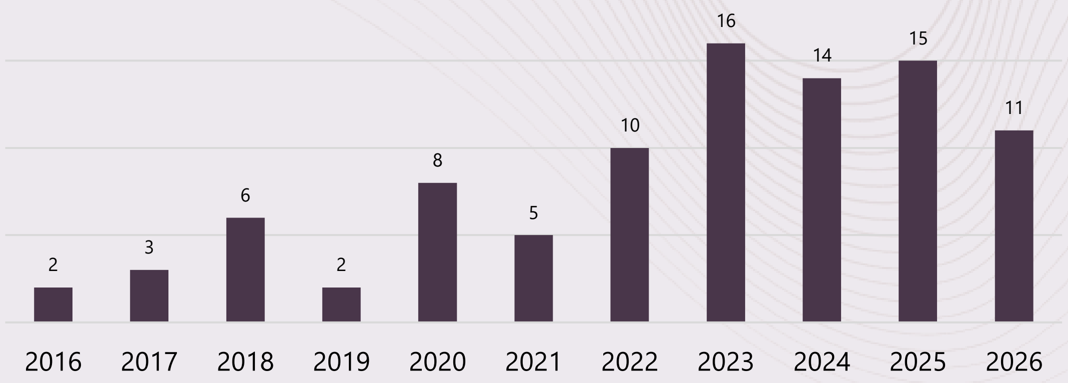
Success Rate across Regulatory Pathways(3/3)

Orphan Medicinal Product Designations (2021-25)



Note: Data taken from Orphan Medicinal Product Designation Overview 2020-2025

Pipeline of Medicines in the PRIME Program (2016–26)



Note: Data as of May 7, 2026 taken from EMA website

KEY HIGHLIGHTS

- ❖ Orphan designations submissions dropped ~21% from 269 (CY2022) to 211 (CY2025) but approval rates rose from ~67% to ~77% for the same period, while withdrawals fell sharply ~61% between CY2023 & CY2025, with EC refusals staying negligible, shows sponsors now file higher-quality, commercially viable rare disease programs
- ❖ PRIME demand continues but is highly selective as designations grew from 2 (CY2016) to a peak of 16 (CY2023) and now stabilize above 10 per year; with <20 PRIME grants annually versus ~200+ orphan designations, underscores that PRIME is reserved for a small number of truly innovative therapies

KEY TAKEAWAY

- ❖ Regulators are increasingly rewarding only the strongest rare disease programs, even as orphan submissions decline, higher approval rates, fewer withdrawals and minimal EC refusals indicate sponsors are self-selecting higher-quality, commercially viable assets, while a much smaller PRIME channel is kept for only the most innovative therapies

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US FDA Novel Drug Approval Landscape

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US FDA Designated Novel Drug Approvals(2025) (1/2)

Accelerated Approval

Oncology	LYNOZYFIC (linvoseltamab-gcpt)	REGENERON	Multiple Myeloma	IV
Oncology	AVMAPKI FAKZYNJA CO-PACK (avutometinib capsules; defactinib tablets) 0.8 mg; 200 mg	VERASTEM ONCOLOGY	Low-grade Serous Ovarian Cancer	PO
Oncology	HYRNUO (sevabertinib)	BAYER	NSCLC	PO
Oncology	KEYTRUDA Qlex (pembrolizumab + berahyaluronidase alfa-pmph)	MERCK	TMB-H Solid Tumors	SC
Oncology	MODEYSO (dordaviprone)	Jazz Pharmaceuticals	Glioma	PO
Oncology	Emrelis (telisotuzumab vedotin-tllv)	abbvie	NSCLC	IV
Oncology	ZEGFROVY 150 mg Tablet	Dizal Pharma	NSCLC	PO
Oncology	HERNEXEOS zongertinib tablets | 60 mg	Boehringer Ingelheim	NSCLC	PO
Rare	Forzinity (elamipretide) injection	Stealth BIOTHERAPEUTICS	Barth Syndrome	SC
Rare	VANRAFIA (atrasentan) tablets	NOVARTIS	IgA Nephropathy	PO
Rare	Voyxact (sibeprenlimab-szsi)	Otsuka	IgA Nephropathy	SC

Breakthrough Therapy Designation

Oncology	AVMAPKI FAKZYNJA CO-PACK (avutometinib capsules; defactinib tablets) 0.8 mg; 200 mg	VERASTEM ONCOLOGY	Low-grade Serous Ovarian Cancer	PO
Oncology	HYRNUO (sevabertinib)	BAYER	NSCLC	PO
Oncology	HERNEXEOS zongertinib tablets | 60 mg	Boehringer Ingelheim	NSCLC	PO
Oncology	ANIKORA (Penpulimab-kcqx)	Akesobio	Nasopharyngeal Carcinoma	IV
Oncology	Emrelis (telisotuzumab vedotin-tllv)	abbvie	NSCLC	IV
Oncology	komzifti (ziftomenib) 200 mg capsules	KYOWA KIRIN, KURA ONCOLOGY	Acute Myeloid Leukemia	PO
Oncology	IBTROZI taletrectinib	Nuvation Bio	NSCLC	PO
Oncology	ZEGFROVY 150 mg Tablet	Dizal Pharma	NSCLC	PO
Other	JOURNAVX (suzetrigine) 50mg tablet	VERTEX	Acute Pain	PO
Other	JASCAYD (nerandomilast)	Boehringer Ingelheim	Idiopathic Pulmonary Fibrosis	PO
Rare	kygevvi	ucb	Thymidine Kinase 2 Deficiency	PO
Rare	MYQORZO (aficamten) tablets	Cytokinetics	oHCM	PO
Rare	Voyxact (sibeprenlimab-szsi)	Otsuka	IgA Nephropathy	SC
Rare	YARTEMLEA	OMEROS	TA-TMA	IV
Rare	Redemplo (plozasiran) injection	arrowhead pharmaceuticals	Familial Chylomicronemia Syndrome	SC

Abbreviations: oHCM: Obstructive Hypertrophic Cardiomyopathy; TMB-H: High Tumor Mutational Burden; TA-TMA: Transplant-associated Thrombotic Microangiopathy; NSCLC: Non-small Cell Lung Cancer

US FDA Designated Novel Drug Approvals(2025) (2/2)

Priority Review

Area	Drug	Company	Route	Indication
Oncology	LYNOZYFIC (linvoseltamab-gcpt)	REGENERON	IV	Multiple Myeloma
Oncology	AVMAPKI FAKZYNJA CO-PACK (avutometinib capsules; defactinib tablets)	VERASTEM ONCOLOGY	PO	Low-grade Serous Ovarian Cancer
Oncology	HYRNUO (sevabertinib)	BAYER	PO	NSCLC
Oncology	Romvimza (vimseltinib)	ono	PO	Tenosynovial Giant Cell Tumor
Oncology	MODEYSO (dordaviprone)	Jazz Pharmaceuticals	PO	Glioma
Oncology	IBTROZI taletrectinib	Nuvation Bio	PO	NSCLC
Oncology	ZEGFROVY 150 mg Tablet	Dizal Pharma	PO	NSCLC
Oncology	HERNEXEOS zongertinib tablets | 60 mg	Boehringer Ingelheim	PO	NSCLC
Oncology	Emrelis (telisotuzumab vedotin-tllv)	abbvie	IV	NSCLC
Oncology	komzifti (ziftomenib)	Kyowa Kirin; KURA ONCOLOGY	PO	Acute Myeloid Leukemia
Other	BLUJEPA (gepotidacin)	GSK	PO	Urinary Tract Infection
Other	NUZOLVENCE (zoliflodacin)	INNOVIVA Specialty Therapeutics	PO	Urogenital Gonorrhea
Other	Brinsupri (brensocatib)	insmed	PO	Non-cystic Fibrosis Bronchiectasis
Other	JASCAYD (nerandomilast)	Boehringer Ingelheim	PO	Idiopathic Pulmonary Fibrosis
Other	JOURNAVX (suzetrigine) 50mg tablet	VERTEX	PO	Acute Pain
Rare	Forzinity (elamipretide) injection	Stealth BIOTHERAPEUTICS	SC	Barth Syndrome
Rare	imaavy (nipocalimab-aahu)	Johnson&Johnson	IV	gMG
Rare	Voyxact (sibeprenlimab-szsi)	Otsuka	SC	IgA Nephropathy
Rare	YARTEMLEA (narsoplimab-wuug)	OMEROS	IV	TA-TMA
Rare	kygevvi	ucb	PO	Thymidine Kinase 2 Deficiency
Rare	GOMEKLI (mirdametinib)	MERCK	PO	NF1-associated PN

Fast Track Designation

Area	Drug	Company	Route	Indication
Oncology	LYNOZYFIC (linvoseltamab-gcpt)	REGENERON	IV	Multiple Myeloma
Oncology	Inluriyo (imlunestrant)	Lilly	PO	Breast Cancer
Oncology	HERNEXEOS zongertinib tablets | 60 mg	Boehringer Ingelheim	PO	NSCLC
Oncology	ANIKORA (Penpulimab-kcqx)	Akesobio	IV	Nasopharyngeal Carcinoma
Oncology	Romvimza (vimseltinib)	ono	PO	Tenosynovial Giant Cell Tumor
Oncology	komzifti (ziftomenib)	Kyowa Kirin; KURA ONCOLOGY	PO	Acute Myeloid Leukemia
Other	JOURNAVX (suzetrigine) 50mg tablet	VERTEX	PO	Acute Pain
Other	ENFLONSIA (clesrovimab-cfor)	MSD	IM	Respiratory Syncytial Virus
Other	NUZOLVENCE (zoliflodacin)	INNOVIVA Specialty Therapeutics	PO	Urogenital Gonorrhea
Other	Qfitlia (fitusiran)	sanofi	SC	Reduce bleeding episodes in Hemophilia A or B
Other	Anzupgo (delgocitinib) cream 2%	LEO	Topical Cream	Chronic Hand Eczema
Rare	ANDEMBRY garadacimab-gxii	CSL Behring	SC	Hereditary Angioedema
Rare	Forzinity (elamipretide) injection	Stealth BIOTHERAPEUTICS	SC	Barth Syndrome
Rare	imaavy (nipocalimab-aahu)	Johnson&Johnson	IV	gMG
Rare	WAYRILZ (rilzabrutinib)	sanofi	PO	Immune Thrombocytopenia
Rare	Redemplo (plozasiran) injection	arrowhead pharmaceuticals	SC	Familial Chylomicronemia Syndrome
Rare	Ekterly (sebetralstat) tablets 300 mg	KalVista; Chiesi	PO	Hereditary Angioedema
Rare	GOMEKLI (mirdametinib)	MERCK	PO	NF1-associated PN

Abbreviations: gMG: Generalized Myasthenia Gravis; NF1: Neurofibromatosis Type 1; PN: Plexiform Neurofibromas; TA-TMA: Transplant-associated Thrombotic Microangiopathy; NSCLC: Non-small Cell Lung Cancer

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Regulatory Designations as a Strategic Asset

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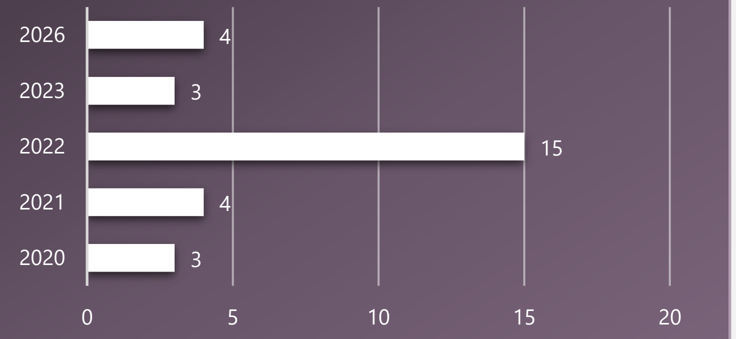
Pfizer Pipeline Regulatory Designation Dashboard (Jan 2020-May 2026)

5	6	7	1	7	2	1
BTD	FTD	ODD	PRIME	PRIORITY REVIEW	RPD	AA

COMPANY OVERVIEW

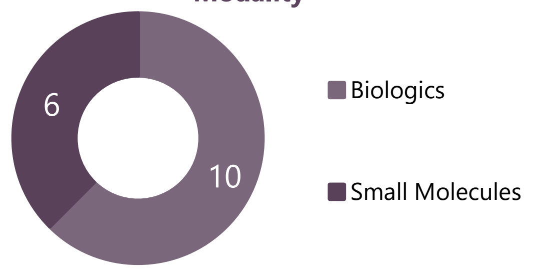
Head Quarter	New York, US
Pipeline Focus	Oncology Inflammation & Immunology Neurology Respiratory Cardiovascular Gastro Intestinal Vaccines
FY Revenue 2025	$62.6B

Distribution of Designations by Year

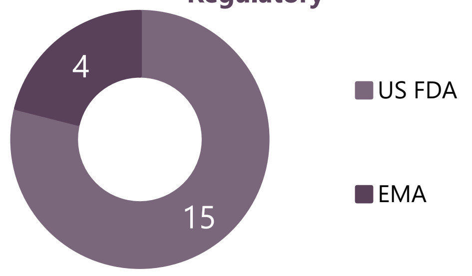


Note: Designation disclosed by company and other sources are included
Data from Jan 2020 - May 2026

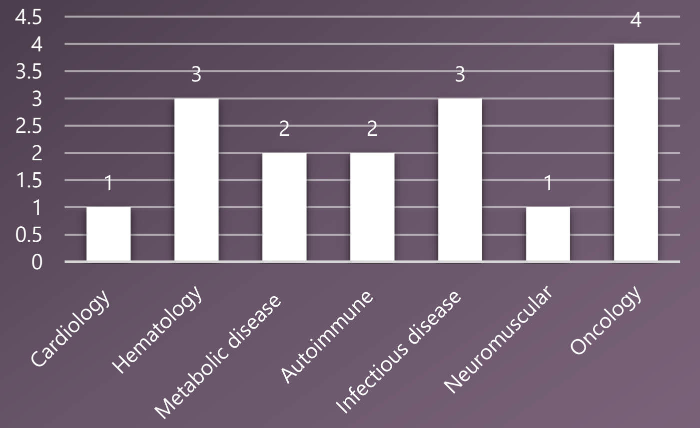
Distribution of Designated Molecules by Modality



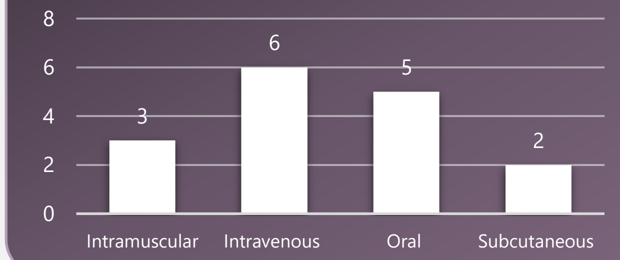
Distribution of Designated Molecules by Regulatory



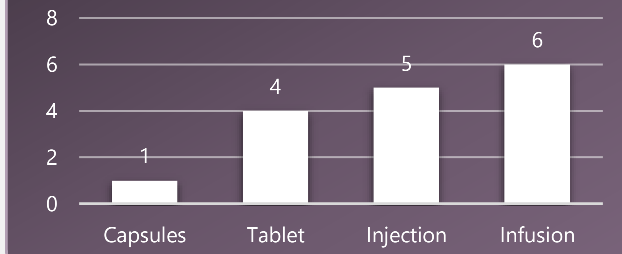
Distribution of Designated Molecules by Therapy Area



Distribution of Designated Molecules by Route of Administration



Distribution of Designated Molecules by Formulation



STRATEGIC INSIGHT

- Pfizer prioritizes Oncology and Infectious diseases, reflecting a focus on high unmet need, high-growth markets
- US FDA designations dominate the portfolio, highlighting a strong US first regulatory strategy
- A balanced biologics and small-molecule mix support diversified innovation and risk management

INSIGHTS CORNER

How Each Designation Serves Company Strategy?

Stacking ODD, FTD, BTD and Priority Review transforms a program into a strategic asset that accelerates time to market, amplifies exclusivity and tax benefits, and boosts investor and M&A value while de-risking FDA review

Competitive moat (ODD + BTD + PR)

ODD exclusivity prevents US FDA from approving a competitor's same drug for the same indication for 7 years

BTD combined with PR can get to market 2-4 years before a competitor on the standard pathway, locking in prescriber habits and payer formularies before the competition even files

Financial strategy (ODD)

The 25% tax credit on clinical trial costs and 7-year exclusivity can be worth $100M+ over a product's lifecycle

Investor and M&A (BTD)

BTD alone raises acquisition premiums by roughly 20-40%. Stacked designations function as a regulatory "seal of quality" that de-risks assets for large pharma acquirers

BTD + ODD programs consistently command 2 to 3× higher IPO valuations than no-designation programs in the same indication

Operational (FTD + BTD)

FTD rolling review means US FDA has already reviewed 40-60% of the NDA before the formal filing date, compressing the real review timeline far beyond what the 6-month PR clock suggests

BTD makes US FDA a co-developer: cross-disciplinary team assignments, senior reviewer access, and collaborative trial design that dramatically reduces the risk of a Complete Response Letter at submission

Accelerating Oncology Approval: How TRODELVY's Designations Cut the Timeline to Commercialization

GILEAD Immunomedics
TRODELVY® sacituzumab govitecan-hziy 180 mg for injection

Designations → Approval Timeline

TRODELVY demonstrates how US FDA expedited designations can transform strong early efficacy data into rapid market entry, while preserving regulatory rigor through mandatory post-approval confirmatory studies

Key Market Access Takeaway:

TRODELVY illustrates how the combination of BTD + PR + AA can shorten the path from promising P2 data to commercialization in oncology.
However, it also highlights the regulatory risk of accelerated approvals, indications can be withdrawn if confirmatory trials fail to verify clinical benefit, as occurred in Urothelial cancer

Approved Indications: ≥2L mTNBC and Pre-Treated HR+/HER2- mBC
Pipeline: 1L TNBC (Filed), 1L NSCLC (P3), SCLC (P3), 2L mEC (P3)

	Indication	Regulatory Milestone	How Each Designation Helped TRODELVY
Pre-2020	**Fast Track Designation**: TNBC; NSCLC; mUC; SCLC **Breakthrough Therapy Designation**: mTNBC		FTD and BTD helped speed up development with more frequent US FDA interactions and faster alignment
Apr 2020	mTNBC after ≥2 prior therapies	**Accelerated approval** based on ORR and DoR in P2 study	First US FDA-approved anti-Trop-2 ADC. Early commercial launch in a high-unmet-need TNBC setting
Apr 2021	mTNBC after ≥2 prior therapies	Conversion from **accelerated to regular approval**	Conversion based on confirmatory P3 ASCENT data
Apr 2021	Pre-Treated mUC	2nd **accelerated approval** (P2 TROPHY study)	This review used the **RTOR pilot program** and the **Assessment Aid, a voluntary submission** from the applicant to facilitate the US FDA's assessment. The US FDA approved this application 6 weeks ahead
Feb 2023	Pre-Treated HR+/HER2- mBC	Regular approval under **Project Orbis,** FDA collaborated with Australian TGA, Health Canada and Switzerland's Swissmedic (PR for sBLA accepted on Oct 2022)	Approval in less than 6 months. Expansion from TNBC into a larger HR+/HER2− market
Oct 2024	Pre-Treated mUC	Urothelial indication **withdrawn** after **failed confirmatory study**	Demonstrates that accelerated approval is conditional and requires verification of clinical benefit

Note: TRODELVY also has BTD for 2L ES-SCLC in Dec 2024; ODD for SCLC and Pancreatic cancer; First BLA in TNBC submitted in 2018 which was followed by a CRL and BLA was re-submitted in 2019

Abbreviations: mTNBC: Metastatic Triple-negative Breast Cancer; mUC: Metastatic Urothelial Cancer; FTD: Fast Track Designation; BTD: Breakthrough Therapy Designation; PR: Priority Review; AA: Accelerated Approval; NSCLC: Non-Small Cell Lung Cancer

Regulatory Developments Impacting Designations

US & EMA Regulatory Reforms:

Next-Generation Regulatory Incentives: How 2026 US FDA and EMA Reforms Redefine Rare, PRIME and Psychedelic Pipelines

Orphan Drug Catalyst Precedent Overturned [Section 6605]

FDA

Feb 3, 2026

The Change: Direct statutory reversal of the 11th Circuit's *Catalyst v. Becerra* (2021) decision

The Shift: Replaces *"same disease or condition"* with *"same approved use or indication within such rare disease."* * **The Impact:** Codifies the US FDA's long-standing, **indication-specific** practice. Broad early orphan designations can no longer completely block competitors from developing therapies for distinct sub-populations

Retroactive Mandate: The US FDA is legally auditing all active orphan exclusivities to see which competing pipelines are cleared for entry (eg: Pasithea's PAS-004 for ALS; TearSolutions Lacripep)

Rare Pediatric Disease PRV Program Reauthorization [Section 6604]

Feb 3, 2026

FDA

The Framework: Completely reverses the Dec 2024 program sunset

New Deadline: Sponsors securing US FDA approval for qualifying pediatric rare diseases before Sep 30, 2029 receive a fully transferable Priority Review Voucher (PRV)

Market Dynamics: Vouchers compress subsequent US FDA drug reviews from 12 to 6 months and command an immediate $100M-$200M market value, offering key non-dilutive capital to biotech pipelines (eg: Latus Bio's LTS-101 for CLN2)

Permanent Integration of PRIME Scheme Upgrades

Mar 18, 2026

EMA
EUROPEAN MEDICINES AGENCY

The Background: Historically, 50% of high-priority PRIME products lost accelerated status and reverted to 210-day reviews due to immature dossiers

The Solutions (Permanently integrated into EMA protocol):

Regulatory Roadmap & Tracker: Replaces sluggish annual reporting with dynamic, real-time, issue-focused milestone alignment

Expedited Scientific Advice: A fast-track pathway providing immediate turnarounds for mid-cycle trial alterations or unexpected CMC (Chemistry, Manufacturing, and Controls) hurdles

Submission Readiness Meeting (SRM): Mandatory session held 9 to 12 months pre-filing with CHMP/CAT Rapporteurs to lock down GCP/GMP compliance, securing the coveted 150-day Accelerated Assessment

Psychedelic Drug Prioritization Policy

Apr 18, 2026

FDA

Executive Fast-Track: Executive order establishes a formal policy to accelerate research and clinical approvals for psychedelic drugs targeting serious mental illnesses

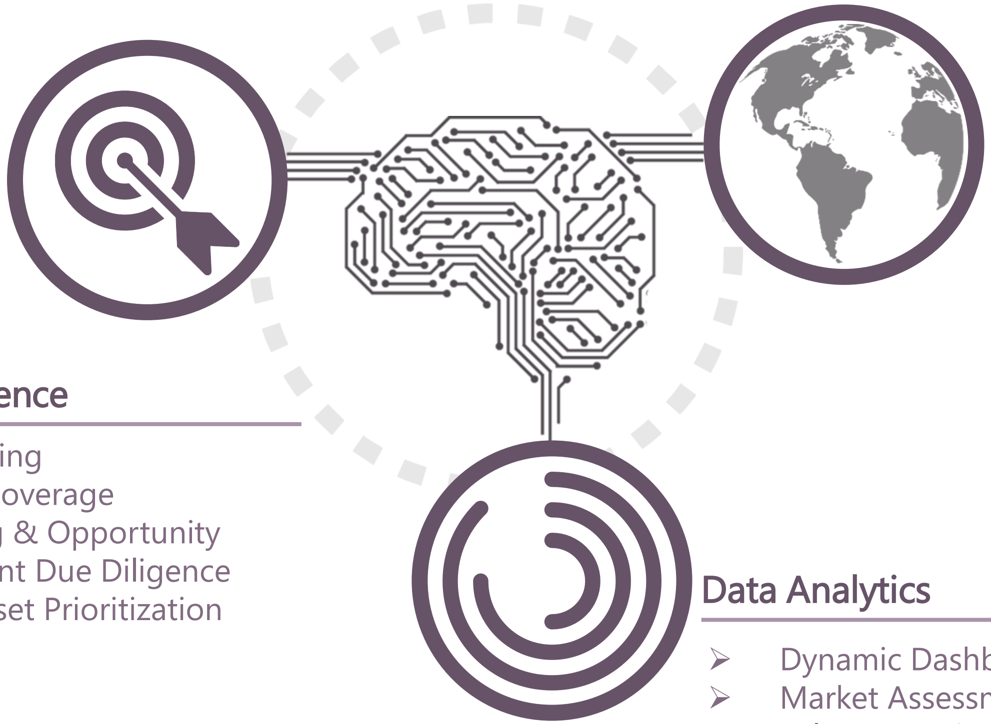
Mandatory Voucher Allocation: Mandates the US FDA to proactively award ultra-fast Commissioner's National Priority Vouchers (CNPVs) to psychedelic compounds holding Breakthrough Therapy designation

Federal Clinical Funding: Directs HHS (Department of Health and Human Services) to allocate at least $50M through ARPA-H to partner with state governments in advancing clinical trial pipelines for these modalities

Abbreviations: CHMP: Committee for Medicinal Products for Human Use; CAT: Committee for Advanced Therapies; GCP: Good Clinical Practice; GMP: Good Manufacturing Practice; ALS: Amyotrophic Lateral Sclerosis; ARPA-H: Advanced Research Projects Agency for Health

ABOUT COMPANY

Octavus delivers evidence-based consulting services to empower our clients in making well-informed decisions.



Market Research

- Quantitative & Qualitative Research
- Awareness, Trial & Usage (ATU) Studies
- KOL & PAG Identification & Mapping
- Concept/Product Testing
- AI & Human Confluence Transcription
- Content Analysis & Translation

Business Intelligence

- CI & Monitoring
- Conference Coverage
- Market Sizing & Opportunity
- Pre-investment Due Diligence
- Company/Asset Prioritization

Data Analytics

- Dynamic Dashboarding & Reporting
- Market Assessment & Epidemiology Analysis
- Sales Reporting & In-line Forecasting
- Social Media Analytics
- Clinical Analytics

OCTAVUS CONSULTING
Data.Insights.Decisions.

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